

## **1. Administrative & Study Identification**

**Official Title:** MAGNETISMM-3 AN OPEN-LABEL, MULTICENTER, NON-RANDOMIZED PHASE 2 STUDY OF ELRANATAMAB (PF-06863135) MONOTHERAPY IN PARTICIPANTS WITH MULTIPLE MYELOMA WHO ARE REFRACTORY TO AT LEAST ONE PROTEASOME INHIBITOR, ONE IMMUNOMODULATORY DRUG AND ONE ANTI-CD38 ANTIBODY **Study Title:** MagnetisMM-3: Study Of Elranatamab (PF-06863135) Monotherapy in Participants With Multiple Myeloma Who Are Refractory to at Least One PI, One IMiD and One Anti-CD38 mAb **Protocol Number:** C1071003 **NCT Number:** NCT04649359 **Posting ID:** 919500182537625564 **Sponsor/Company:** Pfizer Inc. **Investigational Product:** Elranatamab (PF-06863135 / Elrexfio®) - BCMA-CD3 bispecific antibody **Phase of Development:** Phase 2 **Trial Status:** Active, not recruiting **Medical Monitor:** Pfizer Clinical Operations Lead

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## **2. Study Synopsis & Rationale**

**2.1 Study Objective Primary Objective:** The purpose of the study is to evaluate whether single-agent elranatamab (PF-06863135) can provide clinical benefit (specifically, objective response rate) in participants with relapsed/refractory multiple myeloma.

**Secondary Objectives:** To evaluate Progression-Free Survival (PFS), Overall Survival (OS), Duration of Response (DOR), Time to Response (TTR), Minimal Residual Disease (MRD) negativity rate, and the overall safety and tolerability profile, including Objective Response Rate in specific patient sub-cohorts.

**2.2 Background & Rationale** Patients with relapsed/refractory multiple myeloma (RRMM) who have exhausted standard treatment options—specifically those refractory to at least one proteasome inhibitor (PI), one immunomodulatory drug (IMiD), and one anti-CD38 monoclonal antibody—have a poor prognosis. Elranatamab is a bispecific antibody designed to bind to BCMA-expressing multiple myeloma cells and CD3-expressing T-cells, causing targeted T-cell-mediated cytotoxicity to address this critical unmet medical need.

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## **3. Study Design & Methodology**

**3.1 Design Framework Study Type:** Interventional **Control Type:** Single-arm (Non-randomized) **Blinding:** Open-label **Randomization:** N/A

**3.2 Planned Enrollment & Duration Target \$N\$:** 187 participants  
**Study Cohorts:** \* **Cohort A:** Participants who have not received

prior BCMA-directed therapy. \* **Cohort B:** Participants who have received prior BCMA-directed therapy (ADC or CAR T cells). **Number of Sites:** Multicenter, global (including United States, Australia, Belgium, Canada, France, Germany, Japan, Poland, Spain, United Kingdom) **Estimated Study Start:** 02-Feb-2021 **Estimated Primary Completion:** 17-Jun-2022

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## 4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Diagnosis of multiple myeloma (IMWG criteria, Rajkumar et al, 2014). 2. Measurable disease defined by at least one of the following: \* Serum M-protein  $\geq 0.5$  g/dL by SPEP; \* Urinary M-protein excretion  $\geq 200$  mg/24 hours by UPEP; \* Serum immunoglobulin FLC  $\geq 10$  mg/dL ( $\geq 100$  mg/L) AND abnormal serum immunoglobulin kappa to lambda FLC ratio. 3. Disease refractory to at least one PI, refractory to at least one IMiD, and refractory to at least one anti-CD38 antibody. 4. Relapsed/refractory to the last anti-myeloma regimen. 5. Assigned to proper cohort (Cohort A: has not received prior BCMA-directed therapy; Cohort B: has received prior BCMA-directed therapy (ADC or CAR T cells)). 6. ECOG performance status  $\leq 2$ . 7. Resolved acute effects of any prior therapy to baseline severity or CTCAE Grade  $\leq 1$ . 8. Not pregnant and willing to use contraception.

**4.2 Exclusion Criteria** 1. Smoldering multiple myeloma, active plasma cell leukemia, amyloidosis, or POEMS syndrome. 2. Stem cell transplant within 12 weeks prior to enrollment. 3. Active HBV, HCV, SARS-CoV-2, HIV, or any active, uncontrolled bacterial, fungal, or viral infection. 4. Any other active malignancy within 3 years prior to enrollment, except for adequately treated basal cell or squamous cell skin cancer, or carcinoma in situ. 5. Previous administration with an investigational drug within 30 days or 5 half-lives preceding the first dose of study intervention used in this study (whichever is longer). Concomitant administration of other anti-myeloma therapies.

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## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** Subcutaneous (SC) elranatamab administered in 28-day cycles. Treatment begins with step-up priming doses of 12 mg on Cycle 1 Day 1 (C1D1) and 32 mg on C1D4, followed by the full dose of 76 mg once weekly (QW) starting C1D8. After six cycles, persistent responders switch to biweekly dosing (Q2W). **Route of Administration:** Subcutaneous injection **Duration of Treatment:** Continued until disease progression or unacceptable toxicity.

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care for managing multiple myeloma symptoms and mitigating adverse events, including management protocols for Cytokine Release Syndrome (CRS). **Prohibited:** Previous

administration of an investigational drug within 30 days or 5 half-lives preceding the first dose. Concomitant administration of other anti-myeloma therapies.

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## 6. Study Timeline & Visit Schedule

| Visit ID        | Window (Days) | Key Activities/Procedures                                              |
|-----------------|---------------|------------------------------------------------------------------------|
| Screening       | Day -28 to 0  | Informed Consent, Medical History, Baseline Labs (SPEP/UPEP/FLC), ECG  |
| Baseline        | C1 Day 1      | First step-up dose (12 mg SC), Safety Labs, Observation for CRS        |
| Interim 1       | C1 Days 4 & 8 | Second step-up dose (32 mg SC) on D4; First full dose (76 mg SC) on D8 |
| Maintenance     | Cycle 2 to 6  | Weekly (QW) administration, Safety & Efficacy Labs (IMWG criteria)     |
| Dose Transition | Cycle 7+      | Switch to Biweekly (Q2W) dosing for responders, Efficacy Monitoring    |

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## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** Objective Response Rate (ORR) by Blinded Independent Central Review (BICR) as Per International Myeloma Working Group (IMWG) Criteria. Defined as the % of participants with a best overall response of confirmed stringent complete response (sCR), CR, very good partial response (VGPR) or PR per IMWG criteria. **Measurement Method:** Assessed using standard IMWG criteria by Blinded Independent Central Review (BICR).

**7.2 Secondary & Exploratory Endpoints**

- \* Cohort A Only: Objective Response Rate as Per IMWG Criteria by BICR for Participants With Extramedullary Disease (EMD) at Baseline.
- \* Cohort A Only: Objective Response Rate as Per IMWG Criteria by BICR for Participants Without EMD at Baseline.
- \* Duration of Response (DOR) as Per IMWG Criteria by BICR.
- \* Progression-Free Survival (PFS) and Overall Survival (OS).
- \* Time to Response (TTR) and Minimal Residual Disease (MRD) negativity rate (evaluated per IMWG sequencing criteria at a  $10^{-5}$  threshold).

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## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Continuous monitoring focused on common and severe AEs including Cytokine Release Syndrome (CRS), infections, anemia, and neutropenia. **Serious Adverse Events (SAEs):** Reported within standard 24-hour global pharmacovigilance timelines. Step-up dosing regimens specifically designed to mitigate the incidence and severity of CRS. **Data Monitoring Committee (DMC):** Utilized to monitor safety signals, particularly stopping rules regarding treatment-related neurologic events and Grade 3/4 CRS.

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## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Intent-to-Treat (ITT) population for primary efficacy (ORR); Safety Analysis Set for safety parameters (AEs, dose modifications). **Sample Size Justification:** Cohort sizes determined to provide adequate statistical power to evaluate significant ORR improvements over historical baseline response rates in triple-class refractory MM populations. **Handling of Missing Data:** Standard survival analysis methodologies including censoring for PFS and OS endpoints.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard risk-based onsite and remote monitoring to ensure protocol adherence and safety. **Audit Trail:** Validated eCRF capturing systems with comprehensive audit trails for lab data (SPEP, UPEP) and BICR assessments.

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## 11. Study Identifiers & Governance

**Sponsor/Lead Organization:** Pfizer Inc. **Posting ID:** 919500182537625564 **NCT Number:** NCT04649359 **Protocol Number:** C1071003 **Trial Status:** ACTIVE\_NOT\_RECRUITING **Study Title:** MagnetisMM-3: Study Of Elranatamab (PF-06863135) Monotherapy in Participants With Multiple Myeloma Who Are Refractory to at Least One PI, One IMiD and One Anti-CD38 mAb **Official Title:** MAGNETISMM-3 AN OPEN-LABEL, MULTICENTER, NON-RANDOMIZED PHASE 2 STUDY OF ELRANATAMAB (PF-06863135) MONOTHERAPY IN PARTICIPANTS WITH MULTIPLE MYELOMA WHO ARE REFRACTORY TO AT LEAST ONE PROTEASOME INHIBITOR, ONE IMMUNOMODULATORY DRUG AND ONE ANTI-CD38 ANTIBODY

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## 12. Clinical Framework (Multiple Myeloma Specific)

**Disease Area / Primary Condition:** Multiple Myeloma / Refractory multiple myeloma **Diagnosis Threshold:** Refractory to at least one PI, one IMiD, and one anti-CD38 mAb **Medical Methodology:** BCMA-directed T-cell engaging immunotherapy **Optimization Target:** Targeted T-cell-mediated cytotoxicity against BCMA-expressing plasma cells

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## 13. Study Procedures & Methodology

**Management Pathway:** Two step-up priming doses followed by weekly treatment, adjusting to biweekly for persistent responders. **Education Protocol (HCP):** Specialized site training on the

identification, grading, and management of CRS and neurologic toxicities. **Education Protocol (Patient):** Strict guidelines for monitoring fever and neurological symptoms post-infusion. **Quality Audit Mechanism:** Centralized laboratory tracking and Blinded Independent Central Review (BICR) for reliable disease response categorizations.

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## 14. Observation & Follow-Up Schedule

**Total Duration:** Maximum treatment duration through study completion (estimated December 2026) **Onsite Visit Cadence:** Weekly visits during Cycles 1-6, transitioning to biweekly (Q2W) at Cycle 7 and beyond **Remote Monitoring Frequency:** Continuous pharmacovigilance tracking **Checklist Review Interval:** Every 28-day cycle for safety and response assessment

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## 15. Sign-off & Approval

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|--------------------------|--------|-----------|---------------|-----|-----|-----|------|
| Role                     | Name   | Signature | Date          | :-- | :-- | :-- | :--  |
| Principal Investigator   | [Name] | _____     | [DD-MMM-YYYY] |     |     |     |      |
| Clinical Operations Lead | [Name] | _____     | [DD-MMM-YYYY] |     |     |     | Lead |
| Statistician             | [Name] | _____     | [DD-MMM-YYYY] |     |     |     |      |